

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Epilepsy Society Annual Meeting (AES) 2025

Meeting period: December 2025 | Author: Nadia Belhadj

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Silvio Prendergast, MD (Silver Creek Medical Group)

A short exchange over the coffee queue. Reported an adult describing double vision on the current step. Mentioned a session summary from a colleague but did not raise any specific finding.

Conversation — Yesenia Vasquez-Lund, MD (Northgate Neurology Partners)

A hallway conversation during the afternoon break. Was pleased to report that one adult who had exhausted the usual combinations is now doing markedly better. Titration scheduling is handled by nursing staff, which the practice finds workable. Referenced a case series someone forwarded in passing.

Conversation — Nadine Sandoval-Reyes, MD (Windrow Neuroscience Center)

A hallway conversation during the afternoon break. Raised Lennox-Gastaut syndrome, where the available regimens have plainly run out. Described how long-standing patients is triaged now compared with before.

Conversation — Saoirse Inverness, MD (Granite Bay Epilepsy Center)

Introduced by a colleague from the same region and talked for a few minutes. Formulary questions varies by payer, which came up as a general frustration. The travel schedule this year came up, and little else. Flagged how families of Lennox-Gastaut patients keep asking what else exists.

Conversation — Fabian Lammermoor, MD (Juniper Flats Neurology Partners)

A short exchange over the coffee queue. Asked about my travel schedule, since visits keep landing on the busiest clinic days. Mostly small talk about the program and the venue. Flagged post-stroke epilepsy, which this clinic sees more of every year.

Conversation — Bastian Underhill, MD (Northgate Neurology Partners)

Stopped to talk on the way out of a session block, with no agenda on either side. Spent some time on long-standing patients and how workup decisions get sequenced. Returned to how families of Lennox-Gastaut patients keep asking what else exists. Described how the pediatric-to-adult transfers is triaged now compared with before.

Conversation — Jaromir Merriwether, MD (Sable Creek Medical Group)

Caught between sessions in the poster hall; the conversation was short and entirely unprompted. Formulary questions varies by payer, which came up as a general frustration. Noted a drug-resistant adult has stayed seizure-free longer than at any point in the chart.

Conversation — Eloise Quintrell, MD (Beacon Hill Neurology Associates)

We spoke standing near the exhibit aisles, briefly. The travel schedule this year came up, and little else. Nothing actionable this visit this time. Asked about safety and confirmed nothing to report. The main hesitation is the burden of slow titration for older patients and their caregivers.

Conversation — Ulrike Wetherby, MD (Juniper Flats Neurology Partners)

Approached me rather than the other way around, which is worth noting. Returned to the gap for Lennox-Gastaut patients, described as the most difficult conversation in this clinic. The new-patient backlog has stretched out since the summer, which shapes how much time there is for anything else.

Follow-ups have been logged in the interaction system.